

Section D Artworks

D3 Package Insert

For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

ESCIPRA 10 (ESCITALOPRAM TABLETS USP 10 mg)

COMPOSITION

Each film coated tablet contains:

Escitalopram Oxalate USP

Equivalent to Escitalopram10 mg

DESCRIPTION

Escitalopram oxalate is an orally administered selective serotonin reuptake inhibitor (SSRI). Chemically, Escitalopram oxalate is described as S-(+)-1-[3-(dimethyl-amino)propyl]-1-(*p*-fluorophenyl)-5-phthalanarbonitrile oxalate. Its molecular formula is $C_{20}H_{21}FN_2O \cdot C_2H_2O_4$, and molecular weight is 414.40.

PRODUCT DESCRIPTION

White to off-white, circular, biconvex, film coated tablet having ML 60 debossed on one side, break-line on opposite side and having notches on either ends of break line.

PHARMACOLOGICAL CLASSIFICATION

Antidepressants, selective serotonin reuptake inhibitors

PHARMACOLOGICAL ACTION:

ATC Code: N 06 AB 10

Pharmacokinetics

Absorption

Absorption is almost complete and independent of food intake. (Mean time to maximum concentration (mean T_{max}) is 4 hours after multiple dosing).

As with racemic citalopram, the absolute bioavailability of escitalopram is expected to be about 80%.

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Distribution

The apparent volume of distribution ($V_{d\beta}/F$) after oral administration is about 12 to 26 L/kg. The plasma protein binding is below 80% for escitalopram and its main metabolites.

Biotransformation

Escitalopram is metabolised in the liver to the demethylated and didemethylated metabolites. Both of these are pharmacologically active. Alternatively, the nitrogen may be oxidised to form the N-oxide metabolite. Both parent substance and metabolites are partly excreted as glucuronides. After multiple dosing the mean concentrations of the demethyl and didemethyl metabolites are usually 28-31% and <5%, respectively, of the escitalopram concentration. Biotransformation of escitalopram to the demethylated metabolite is mediated primarily by CYP2C19. Some contribution by the enzymes CYP3A4 and CYP2D6 is possible.

Elimination

The elimination half-life ($t_{1/2\beta}$) after multiple dosing is about 30 hours and the oral plasma clearance (Cl_{oral}) is about 0.6 L/min. The major metabolites have a significantly longer half-life. Escitalopram and major metabolites are assumed to be eliminated by both the hepatic (metabolic) and the renal routes, with the major part of the dose excreted as metabolites in the urine.

Pharmacodynamics

Mechanism of action

Escitalopram is a selective inhibitor of serotonin (5-HT) re-uptake with high affinity for the primary binding site. It also binds to an allosteric site on the serotonin transporter, with a 1000 fold lower affinity.

Escitalopram has no or low affinity for a number of receptors including 5-HT_{1A}, 5-HT₂, DA D₁ and D₂ receptors, α_1 -, α_2 -, β -adrenoceptors, histamine H₁, muscarine cholinergic, benzodiazepine, and opioid receptors.

The inhibition of 5-HT re-uptake is the only likely mechanism of action explaining the pharmacological and clinical effects of escitalopram.

INDICATIONS AND USAGE

- Treatment of major depressive episodes.
- Treatment of panic disorder with or without agoraphobia.
- Treatment of social anxiety disorder (social phobia).
- Treatment of generalised anxiety disorder.
- Treatment of obsessive-compulsive disorder.

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CONTRAINDICATIONS

- Hypersensitivity to the active substance or to any of the excipients,
- Concomitant treatment with non-selective, irreversible monoamine oxidase inhibitors (MAO-inhibitors) is contraindicated due to the risk of serotonin syndrome with agitation, tremor, hyperthermia etc.
- The combination of escitalopram with reversible MAO-A inhibitors (e.g. moclobemide) or the reversible non-selective MAO-inhibitor linezolid is contraindicated due to the risk of onset of a serotonin syndrome.
- Escitalopram is contraindicated in patients with known QT interval prolongation or congenital long QT syndrome.
- Escitalopram is contraindicated together with medicinal products that are known to prolong the QT interval.
- Concomitant treatment with pimozide.

ADVERSE REACTION

System organ class	Frequency	Undesirable Effect
Blood and lymphatic system disorders	Not known	Thrombocytopenia
Immune system disorders	Rare	Anaphylactic reaction
Endocrine disorders	Not known	Inappropriate ADH secretion
Metabolism and nutrition disorders	Common	Decreased appetite, increased appetite, weight increased
	Uncommon	Weight decreased
	Not known	Hyponatraemia, anorexia ¹
Psychiatric disorders	Common	Anxiety, restlessness, abnormal dreams libido decreased Female: anorgasmia
	Uncommon	Bruxism, agitation, nervousness, panic attack, confusional state

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	Rare	Aggression, depersonalisation, hallucination
	Not known	Mania, suicidal ideation, suicidal behaviour ²
Nervous system disorders	Very common	Headache
	Common	Insomnia, somnolence, dizziness, paraesthesia, tremor
	Uncommon	Taste disturbance, sleep disorder, syncope
	Rare	Serotonin syndrome
	Not known	Dyskinesia, movement disorder, convulsion, psychomotor restlessness/akathisia ¹
Eye disorders	Uncommon	Mydriasis, visual disturbance
Ear and labyrinth disorders	Uncommon	Tinnitus
Cardiac disorders	Uncommon	Tachycardia
	Rare	Bradycardia
	Not known	Electrocardiogram QT prolonged Ventricular arrhythmia including torsade de pointes
Vascular disorders	Not known	Orthostatic hypotension
Respiratory, thoracic and mediastinal disorders	Common	Sinusitis, yawning
	Uncommon	Epistaxis
Gastrointestinal disorders	Very common	Nausea
	Common	Diarrhoea, constipation, vomiting, dry mouth
	Uncommon	Gastrointestinal haemorrhages (including rectal haemorrhage)
Hepatobiliary disorders	Not known	Hepatitis, liver function test abnormal

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Skin and subcutaneous tissue disorders	Common	Sweating increased
	Uncommon	Urticaria, alopecia, rash, pruritus
	Not known	Ecchymosis, angioedemas
Musculoskeletal and connective tissue disorders	Common	Arthralgia, myalgia
Renal and urinary disorders	Not known	Urinary retention
Reproductive system and breast disorders	Common	Male: ejaculation disorder, impotence
	Uncommon	Female: metrorrhagia, menorrhagia
	Not known	Galactorrhoea Male: priapism
General disorders and administration site conditions	Common	Fatigue, pyrexia
	Uncommon	Oedema

1. These events have been reported for the therapeutic class of SSRIs.
2. Cases of suicidal ideation and suicidal behaviours have been reported during escitalopram therapy or early after treatment discontinuation
3. Discontinuation symptoms seen when stopping treatment

Discontinuation of SSRIs/SNRIs (particularly when abrupt) commonly leads to discontinuation symptoms. Dizziness, sensory disturbances (including paraesthesia and electric shock sensations), sleep disturbances (including insomnia and intense dreams), agitation or anxiety, nausea and/or vomiting, tremor, confusion, sweating, headache, diarrhoea, palpitations, emotional instability, irritability, and visual disturbances are the most commonly reported reactions. Generally these events are mild to moderate and are self-limiting, however, in some patients they may be severe and/or prolonged. It is therefore advised that when escitalopram treatment is no longer required, gradual discontinuation by dose tapering should be carried out.

DOSAGE

Escitalopram is administered as a single daily dose and may be taken with or without food.

Major depressive episodes

Usual dosage is 10 mg once daily. Depending on individual patient response, the dose may be increased to a maximum of 20 mg daily.

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Usually 2-4 weeks are necessary to obtain antidepressant response. After the symptoms resolve, treatment for at least 6 months is required for consolidation of the response.

Panic disorder with or without agoraphobia

An initial dose of 5 mg is recommended for the first week before increasing the dose to 10 mg daily. The dose may be further increased, up to a maximum of 20 mg daily, dependent on individual patient response.

Maximum effectiveness is reached after about 3 months. The treatment lasts several months.

Social anxiety disorder

Usual dosage is 10 mg once daily. Usually 2-4 weeks are necessary to obtain symptom relief. The dose may subsequently, depending on individual patient response, be decreased to 5 mg or increased to a maximum of 20 mg daily.

Social anxiety disorder is a disease with a chronic course, and treatment for 12 weeks is recommended to consolidate response. Long-term treatment of responders has been studied for 6 months and can be considered on an individual basis to prevent relapse; treatment benefits should be re-evaluated at regular intervals.

Social anxiety disorder is a well-defined diagnostic terminology of a specific disorder, which should not be confounded with excessive shyness. Pharmacotherapy is only indicated if the disorder interferes significantly with professional and social activities.

The place of this treatment compared to cognitive behavioural therapy has not been assessed.

Pharmacotherapy is part of an overall therapeutic strategy.

Generalised anxiety disorder

Initial dosage is 10 mg once daily. Depending on the individual patient response, the dose may be increased to a maximum of 20 mg daily.

Long-term treatment of responders has been studied for at least 6 months in patients receiving 20 mg daily. Treatment benefits and dose should be re-evaluated at regular intervals.

Obsessive-compulsive disorder

Initial dosage is 10 mg once daily. Depending on the individual patient response, the dose may be increased to a maximum of 20 mg daily.

As OCD is a chronic disease, patients should be treated for a sufficient period to ensure that they are symptom free.

Treatment benefits and dose should be re-evaluated at regular intervals.

Elderly patients (> 65 years of age)

Initial dosage is 5 mg once daily. Depending on individual patient response the dose may be increased to 10 mg daily.

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The efficacy of Escitalopram in social anxiety disorder has not been studied in elderly patients.

Children and adolescents (<18 years)

Escitalopram should not be used in the treatment of children and adolescents under the age of 18 years.

Reduced renal function

Dosage adjustment is not necessary in patients with mild or moderate renal impairment. Caution is advised in patients with severely reduced renal function (CL_{CR} less than 30 ml/min.).

Reduced hepatic function

An initial dose of 5 mg daily for the first two weeks of treatment is recommended in patients with mild or moderate hepatic impairment. Depending on individual patient response, the dose may be increased to 10 mg daily. Caution and extra careful dose titration is advised in patients with severely reduced hepatic function.

Poor metabolisers of CYP2C19

For patients who are known to be poor metabolisers with respect to CYP2C19, an initial dose of 5 mg daily during the first two weeks of treatment is recommended. Depending on individual patient response, the dose may be increased to 10 mg daily.

Discontinuation symptoms

When stopping treatment with escitalopram the dose should be gradually reduced over a period of at least one to two weeks in order to reduce the risk of discontinuation symptoms.

DRUG INTERACTIONS

Pharmacodynamic interactions

Contraindicated combinations:

Irreversible non-selective MAOIs

Cases of serious reactions may occur in patients receiving an SSRI in combination with a non-selective, irreversible monoamine oxidase inhibitor (MAOI), and in patients who have recently discontinued SSRI treatment and have been started on such MAOI treatment. Some patient may develop serotonin syndrome.

Escitalopram is contraindicated in combination with non-selective, irreversible MAOIs. Escitalopram may be started 14 days after discontinuing treatment with an irreversible MAOI. At least 7 days should elapse after discontinuing escitalopram treatment, before starting a non-selective, irreversible MAOI.

Reversible, selective MAO-A inhibitor (moclobemide)

Due to the risk of serotonin syndrome, the combination of escitalopram with a MAO-A inhibitor such as moclobemide is contraindicated. If the combination proves necessary, it should be started at the minimum recommended dosage and clinical monitoring should be reinforced.

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Reversible, non-selective MAO-inhibitor (linezolid)

The antibiotic linezolid is a reversible non-selective MAO-inhibitor and should not be given to patients treated with escitalopram. If the combination proves necessary, it should be given with minimum dosages and under close clinical monitoring.

Irreversible, selective MAO-B inhibitor (selegiline)

In combination with selegiline (irreversible MAO-B inhibitor), caution is required due to the risk of developing serotonin syndrome. Selegiline doses up to 10 mg/day have been safely co-administered with racemic citalopram.

QT interval prolongation

Co-administration of escitalopram with medicinal products that prolong the QT interval, such as Class IA and III antiarrhythmics, antipsychotics (e.g. phenothiazine derivatives, pimozide, haloperidol), tricyclic antidepressants, certain antimicrobial agents (e.g. sparfloxacin, moxifloxacin, erythromycin IV, pentamidine, anti-malarial treatment particularly halofantrine), certain antihistamines (e.g. astemizole, mizolastine), is contraindicated.

Combinations requiring precautions for use:

Serotonergic medicinal products

Co-administration with serotonergic medicinal products (e.g. tramadol, sumatriptan and other triptans) may lead to serotonin syndrome.

Medicinal products lowering the seizure threshold

SSRIs can lower the seizure threshold. Caution is advised when concomitantly using other medicinal products capable of lowering the seizure threshold (e.g. antidepressants (tricyclics, SSRIs), neuroleptics (phenothiazines, thioxanthenes and butyrophenones), mefloquin, bupropion and tramadol).

Lithium, tryptophan

There may be enhanced effects when SSRIs is given together with lithium or tryptophan, therefore concomitant use of SSRIs with these medicinal products should be undertaken with caution.

St. John's wort

Concomitant use of SSRIs and herbal remedies containing St. John's wort (*Hypericum perforatum*) may result in an increased incidence of adverse reactions.

Haemorrhage

Altered anti-coagulant effects may occur when escitalopram is combined with oral anticoagulants. Patients receiving oral anticoagulant therapy should receive careful coagulation monitoring when escitalopram is started or stopped. Concomitant use of non-steroidal anti-inflammatory drugs (NSAIDs) may increase bleeding-tendency.

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Alcohol

No pharmacodynamic or pharmacokinetic interactions are expected between escitalopram and alcohol. However, as with other psychotropic medicinal products, the combination with alcohol is not advisable.

Medicinal products inducing hypokalaemia/ hypomagnesaemia

Caution is warranted for concomitant use of hypokalaemia/hypomagnesaemia inducing medicinal products as these conditions increase the risk of malignant arrhythmias.

Pharmacokinetic interactions

Influence of other medicinal products on the pharmacokinetics of escitalopram

The metabolism of escitalopram is mainly mediated by CYP2C19. CYP3A4 and CYP2D6 may also contribute to the metabolism although to a smaller extent. The metabolism of the major metabolite S-DCT (demethylated escitalopram) seems to be partly catalysed by CYP2D6.

Omeprazole: Co-administration of escitalopram with omeprazole (a CYP2C19 inhibitor) results in moderate increase in the plasma concentrations of escitalopram.

Cimetidine: Co-administration of escitalopram with cimetidine (moderately potent general enzyme-inhibitor) may result in a moderate increase in the plasma concentrations of escitalopram. Caution is advised when administering escitalopram in combination with cimetidine. Dose adjustment may be warranted.

Thus, caution should be exercised when used concomitantly with CYP2C19 inhibitors (e.g. omeprazole, esomeprazole, fluvoxamine, lansoprazole, ticlopidine) or cimetidine. A reduction in the dose of escitalopram may be necessary based on monitoring of side-effects during concomitant treatment.

Effect of escitalopram on the pharmacokinetics of other medicinal products

Escitalopram is an inhibitor of the enzyme CYP2D6. Caution is recommended when escitalopram is co-administered with medicinal products that are mainly metabolised by this enzyme, and that have a narrow therapeutic index, e.g. flecainide, propafenone and metoprolol (when used in cardiac failure), or some CNS acting medicinal products that are mainly metabolised by CYP2D6, e.g. antidepressants such as desipramine, clomipramine and nortriptyline or antipsychotics like risperidone, thioridazine and haloperidol. Dosage adjustment may be warranted.

Co-administration with desipramine or metoprolol may result in both cases in a two fold increase in the plasma levels of these two CYP2D6 substrates.

Escitalopram may also cause weak inhibition of CYP2C19. Caution is recommended with concomitant use of medicinal products that are metabolised by CYP2C19.

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WARNINGS & PRECAUTIONS

The following special warnings and precautions apply to the therapeutic class of SSRIs (Selective Serotonin Re-uptake Inhibitors).

Use in children and adolescents

Escitalopram should not be used in the treatment of children and adolescents under the age of 18 years. Antidepressants increase the risk of suicidal thinking and behavior (suicidality) in children and adolescents with major depressive disorder (MDD) and other psychiatric disorders. Anyone considering the use of an antidepressant in a child or adolescent for any clinical use must balance the risk of increased suicidality with the clinical need. If, based on clinical need, a decision to treat is nevertheless taken, patients who are started on therapy should be observed closely for clinical worsening, suicidality, or unusual changes in behavior. Families and caregivers should be advised to closely observe the patient and to communicate with the prescriber.

Paradoxical anxiety

Some patients with panic disorder may experience increased anxiety symptoms at the beginning of treatment with antidepressants. This paradoxical reaction usually subsides within two weeks during continued treatment. A low starting dose is advised to reduce the likelihood of an anxiogenic effect.

Seizures

Escitalopram should be discontinued if a patient develops seizures for the first time, or if there is an increase in seizure frequency (in patients with a previous diagnosis of epilepsy). SSRIs should be avoided in patients with unstable epilepsy, and patients with controlled epilepsy should be closely monitored.

Mania

SSRIs should be used with caution in patients with a history of mania/hypomania. SSRIs should be discontinued in any patient entering a manic phase.

Diabetes

In patients with diabetes, treatment with an SSRI may alter glycaemic control (hypoglycaemia or hyperglycaemia). Insulin and/or oral hypoglycaemic dosage may need to be adjusted.

Suicide/suicidal thoughts or clinical worsening

Depression is associated with an increased risk of suicidal thoughts, self harm and suicide (suicide-related events). As improvement may not occur during the first few weeks or more of treatment, patients should be closely monitored until such improvement occurs. It is general clinical experience that the risk of suicide may increase in the early stages of recovery.

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Patients with a history of suicide-related events, or those exhibiting a significant degree of suicidal ideation prior to commencement of treatment, are known to be at greater risk of suicidal thoughts or suicide attempts, and should receive careful monitoring during treatment.

Other psychiatric conditions for which Escitalopram is prescribed can also be associated with an increased risk of suicide-related events. In addition, these conditions may be co-morbid with major depressive disorder. The same precautions observed when treating patients with major depressive disorder should therefore be observed when treating patients with other psychiatric disorders.

Patients (and caregivers of patients) should be alerted about the need to monitor for any clinical worsening, suicidal behaviour or thoughts and unusual changes in behaviour and to seek medical advice immediately if these symptoms present.

Akathisia/psychomotor restlessness

The use of SSRIs/SNRIs has been associated with the development of akathisia, characterised by a subjectively unpleasant or distressing restlessness and need to move often accompanied by an inability to sit or stand still. This is most likely to occur within the first few weeks of treatment. In patients who develop these symptoms, increasing the dose may be detrimental.

Hyponatraemia

Hyponatraemia, probably due to inappropriate antidiuretic hormone secretion (SIADH), may occur rarely with the use of SSRIs and generally resolves on discontinuation of therapy. Caution should be exercised in patients at risk, such as the elderly, or patients with cirrhosis, or if used in combination with other medications which may cause hyponatraemia.

Haemorrhage

There may be cutaneous bleeding abnormalities, such as ecchymoses and purpura, with SSRIs. Caution is advised in patients taking SSRIs, particularly in concomitant use with oral anticoagulants, with medicinal products known to affect platelet function (e.g. atypical antipsychotics and phenothiazines, most tricyclic antidepressants, acetylsalicylic acid and non-steroidal anti-inflammatory medicinal products (NSAIDs), ticlopidine and dipyridamole) and in patients with known bleeding tendencies.

ECT (electroconvulsive therapy)

Caution is advisable during the concurrent administration of SSRIs and ECT.

Serotonin syndrome

Caution is advisable if escitalopram is used concomitantly with medicinal products with serotonergic effects such as sumatriptan or other triptans, tramadol and tryptophan.

A combination of symptoms, such as agitation, tremor, myoclonus and hyperthermia may indicate the development of this condition. If this occurs treatment with the SSRI and the serotonergic medicinal product should be discontinued immediately and symptomatic treatment initiated.

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St. John's wort

Concomitant use of SSRIs and herbal remedies containing St. John's wort (*Hypericum perforatum*) may result in an increased incidence of adverse reactions.

Discontinuation symptoms seen when stopping treatment

Discontinuation symptoms when stopping treatment are common, particularly if discontinuation is abrupt. The risk of discontinuation symptoms may be dependent on several factors including the duration and dose of therapy and the rate of dose reduction. Dizziness, sensory disturbances (including paraesthesia and electric shock sensations), sleep disturbances (including insomnia and intense dreams), agitation or anxiety, nausea and/or vomiting, tremor, confusion, sweating, headache, diarrhoea, palpitations, emotional instability, irritability, and visual disturbances are the most common reactions. Generally these symptoms are mild to moderate; however, in some patients they may be severe in intensity.

They usually occur within the first few days of discontinuing treatment.

Generally these symptoms are self-limiting and usually resolve within 2 weeks, though in some individuals they may be prolonged (2-3 months or more). It is therefore advised that escitalopram should be gradually tapered when discontinuing treatment over a period of several weeks or months, according to the patient's needs.

Coronary heart disease

Caution is advised in patients with coronary heart disease.

QT interval prolongation

Escitalopram has been found to cause a dose-dependent prolongation of the QT interval. QT interval prolongation and ventricular arrhythmia including torsade de pointes may occur, predominantly in patients of female gender, with hypokalaemia, or with pre-existing QT interval prolongation or other cardiac diseases.

Caution is advised in patients with significant bradycardia; or in patients with recent acute myocardial infarction or uncompensated heart failure.

Electrolyte disturbances such as hypokalaemia and hypomagnesaemia increase the risk for malignant arrhythmias and should be corrected before treatment with escitalopram is started.

If patients with stable cardiac disease are treated, an ECG review should be considered before treatment is started.

If signs of cardiac arrhythmia occur during treatment with escitalopram, the treatment should be withdrawn and an ECG should be performed.

Angle-Closure Glaucoma

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SSRIs including escitalopram may have an effect on pupil size resulting in mydriasis. This mydriatic effect has the potential to narrow the eye angle resulting in increased intraocular pressure and angle-closure glaucoma, especially in patients pre-disposed. Escitalopram should therefore be used with caution in patients with angle-closure glaucoma or history of glaucoma.

Pregnancy & Lactation

Pregnancy:

Animal studies have shown reproductive toxicity. Escitalopram should not be used during pregnancy unless clearly necessary and only after careful consideration of the risk/benefit.

Neonates should be observed if maternal use of Escitalopram continues into the later stages of pregnancy, particularly in the third trimester. Abrupt discontinuation should be avoided during pregnancy.

The following symptoms may occur in the neonate after maternal SSRI/SNRI use in later stages of pregnancy: respiratory distress, cyanosis, apnoea, seizures, temperature instability, feeding difficulty, vomiting, hypoglycaemia, hypertonia, hypotonia, hyperreflexia, tremor, jitteriness, irritability, lethargy, constant crying, somnolence and difficulty sleeping. These symptoms could be due to either serotonergic effects or discontinuation symptoms. In a majority of instances the complications begin immediately or soon (<24 hours) after delivery.

Epidemiological data have suggested that the use of SSRIs in pregnancy, particularly in late pregnancy, may increase the risk of persistent pulmonary hypertension in the newborn (PPHN). The observed risk was approximately 5 cases per 1000 pregnancies. In the general population 1 to 2 cases of PPHN per 1000 pregnancies occur.

Breast-feeding

It is expected that escitalopram will be excreted into human milk. Consequently, breast-feeding is not recommended during treatment.

Observational data indicate an increased risk (less than 2-fold) of postpartum haemorrhage following SSRI/SNRI exposure within the month prior to birth.

OVERDOSAGE

Toxicity

Clinical data on Escitalopram overdose are limited and many cases involve concomitant overdoses of other drugs. In the majority of cases mild or no symptoms have been reported. Fatal cases of Escitalopram overdose have rarely been reported with Escitalopram alone. The majority of cases have involved over dose with concomitant medications. Doses between 400 and 800mg of Escitalopram alone have been taken without any severe symptoms.

Management

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There is no specific antidote. Establish and maintain an airway, ensure adequate oxygenation and respiratory function. Gastric lavage and the use of activated charcoal should be considered. Gastric lavage should be carried out as soon as possible after oral ingestion. Cardiac and vital signs monitoring are recommended along with general symptomatic supportive measures.

ECG monitoring is advised in case of overdose in patients with congestive heart failure/bradyarrhythmias, in patients using concomitant medications that prolong the QT interval, or in patients with altered metabolism, e.g. liver impairment.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE

Although escitalopram has been shown not to affect intellectual function or psychomotor performance, any psychoactive medicinal product may impair judgement or skills. Patients should be cautioned about the potential risk of an influence on their ability to drive a car and operate machinery.

INSTRUCTIONS FOR USE

Escitalopram is administered as a single daily dose and may be taken with or without food.

STORAGE

Store below 30°C in a dry place, protect from light .

KEEP OUT OF REACH OF CHILDREN.

PRESENTATION

Alu/Alu Blister pack of 14 Tablets. Such 2 blisters packed in a carton along with a pack insert.

Alu/Alu Blister pack of 7 Tablets. Such 4 blisters packed in a carton along with a pack insert.

MANUFACTURED BY

MACLEODS PHARMACEUTICALS LTD.

Block N-2, Village Theda,

Post office Lodhimajra,

Tehsil Baddi, District Solan,

Himachal Pradesh, IN-174101, India

Office: Atlanta Arcade, Church Road, Andheri-Kurla Road, Andheri (East)

Mumbai - 400059. INDIA.

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PRODUCT REGISTRATION HOLDER IN MALAYSIA

Synerrv Sdn Bhd,

So-29-2, Menara 1, Kl Eco City, Jalan Bangsar, Kg Haji

Abdullah Hukum, 59200 Kuala Lumpur Wilayah Persekutuan

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